coursera

Depressive and Bipolar Disorders ✕

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Opening to Depressive and Bipolar Disorders

Module 3
Depressive and Bipolar Disorders

Module 4
Biological Contributing Factors

Lesson 5: Genetic Contributing Factors

✓ Developmental Psychopathology
Reading • 3 min

✓ Genetic Contributing Factors
Reading • 3 min

✓ The Link Between Genes and Psychopathology
Video • 4 min

✓ Practical Scenario
Real-world application • 5 min

✓ Genetic Contributing Factors for Depression
Reading • 10 min

✓ Epigenetic Contributors to Depression
Video • 3 min

✓ Genetic Contributing Factors for Bipolar Disorder
Reading • 3 min

○ Check Your Understanding: Genetic Contributing Factors
Practice Assignment • 4 min

Lesson 6: Neurobiological Contributing Factors

✓ Depressive Disorders: A Case Study
Reading • 5 min

✓ Communication in the Synapse
Video • 4 min

✓ Neurochemical Contributors to Depression
Reading • 10 min

✓ Practical Scenario
Real-world application • 5 min

✓ Hormonal Contributors to Depression
Reading • 5 min

✓ Brain Regions Linked to Depression
Video • 4 min

✓ Neurobiological Contributors to Depressive and Bipolar Disorders
Reading • 15 min

✓ Check Your Understanding: Biological Contributing Factors
Practice Assignment • Grade: 83.33%

Key Takeaways

✓ Key Takeaways: Biological Contributing Factors
Reading • 10 min

Module 4 Quiz

✓ Module Quiz: Biological Contributing Factors
Graded Assignment • Grade: 88.88%

Module 5
Psychological Contributing Factors

Lesson 7: Psychological Contributing Factors

✓ Psychological Contributing Factors
Reading • 10 min

✓ Check Your Understanding: Psychological Contributing Factors
Practice Assignment • Grade: 100%

Lesson 8: Stress and Mood Disorders

✓ Stress and Mood Disorders
Reading • 15 min

✓ Check Your Understanding: Stress and Mood Disorders
Practice Assignment • Grade: 100%

Key Takeaways

○ Key Takeaways: Psychological Contributing Factors
Reading • 10 min

Module 5 Quiz

○ Module Quiz: Contributing Factors II
Graded Assignment • 10 min

Module 6
Treatments for Depressive and Bipolar Disorders

Module 7
Suicide

Module 8
Conclusion

Module 9
Cumulative Quiz

Module 10
Course Resources

Module 11
PsycLearn Essentials APA Student Resources

Key Takeaways: Psychological Contributing Factors

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Learning Objectives

This module focused on helping you to be able to:

- Compare and contrast common mood disorders.
  - Describe the psychological contributing factors for mood disorders.
  - Explain the relationship between stress and mood disorders.

Learning objective: Describe the psychological contributing factors for mood disorders.

1. At every stage we experience biological, social, cognitive, emotional, and psychological challenges. Developmental psychopathologists believe it is important to consider all these factors in explaining behavior; no single factor in isolation is responsible for causing psychological disorders.
2. **Automatic thoughts** are instantaneous, habitual, and nonconscious thoughts that may be positive or negative.
3. **Negative automatic thoughts (NATs)** are instantaneous, habitual, and self-critical. When these negative automatic thoughts involve exaggerations, misinterpretations, and distortions of an event then we refer to them as **cognitive distortions**.
4. The **cognitive triad** is part of a person's larger system of beliefs about the self, the world, and the future, and affects how people respond to setbacks and challenges. When all three sets of automatic thoughts are negative, people may feel hopeless and are unmotivated to try to improve their situation.
5. The **generic cognitive model** proposes that psychological disorders result from an extreme version of adaptive thinking. Experiencing a variety of emotions, such as happiness, anger, sadness, and anxiety, is normal and adaptive; however, when people experience emotions to an extreme degree, often because of cognitive biases, they are at risk for psychopathology.
6. The way we think about the world can both cause and reinforce depressive episodes. In addition to negative thoughts about the self, a wide range of cognitive processes influence the onset and course of depression, including attention, interpretation, and memory.
7. Depression has been shown to be associated with dysfunction in attentional processes; specifically, individuals who are depressed appear to have difficulty drawing their attention away from negative stimuli, such as sad faces, to focus on neutral stimuli. This difficulty in shifting attention by people with depression appears to play a role in the maintenance of sad moods.
8. People with depression are more likely to interpret ambiguous information in negative ways. Once depressed people make negative interpretations, those interpretations are encoded in their memory. When combined with the evidence that individuals who are depressed exhibit mood-congruent memory, which means that they remember more negative events and information than nondepressed participants, this creates an interaction where attention to negative stimuli is misinterpreted and then encoded and recalled disproportionately.
9. The impact that our thoughts can have on depression arises because much of what we experience every day is ambiguous. Many events that we experience or think about can be interpreted in a variety of ways. People with depression tend to interpret these ambiguous events more negatively than others.
10. **Helplessness theory** (or learned helplessness) theorizes that when people are not able to exert control in their lives, this may lead to a belief that they are helpless. Feeling helpless has been linked to depressive symptoms, including feelings of sadness, loss of interest in usual activities, changes in sleep and appetite, and feelings of worthlessness.
11. **Hopelessness theory** theorizes that people develop depression if they attributed a negative event to causes that were internal, stable, and global as opposed to blaming it on external, specific, and unstable stimuli. According to the hopelessness theory, these negative cognitive styles interact with negative life events to produce hopelessness and contribute to depression.
12. There is a tendency of people with depression to engage in rumination. **Rumination** is obsessional thinking involving excessive, repetitive thoughts or themes that interfere with other forms of mental activity. People who ruminate remain focused on the problems they are experiencing, rather than on active problem-solving to deal with the problem and prevent it from recurring. Rumination has long been associated with both the beginning and continuation of depressive episodes. The relationship between rumination and depression may be bidirectional. Rumination may predict depression and depression may predict rumination.

Learning objective: Explain the relationship between stress and mood disorders.

1. When stress is prolonged or chronic it might lead to developing depression, bipolar disorder, or other psychological disorders.
2. **Adverse childhood experiences,** which are negative events experienced before the age of 18, are often separated into two categories: child abuse and neglect (including include physical abuse, emotional abuse, sexual abuse, physical neglect, and emotional neglect), and household challenges (exposure to the mother being treated violently, parental divorce or separation, parental incarceration, a household member with substance abuse problems, and a household member with mental illness). Researchers typically find that as the number of adverse childhood experiences a person experiences increases, so too does the likelihood of their experiencing depressive symptoms in adulthood.
3. All subcomponents of childhood trauma (emotional, physical, and sexual abuse) are strongly linked with bipolar disorder, yet there is continued debate on the role of each trauma subtype. Childhood trauma is consistently linked to an earlier age of onset of bipolar disorder, increased risk of suicide attempts, and cooccurring substance misuse.
4. **Toxic stress** is the excessive, prolonged and/or frequent activation of the stress response systems in the body and brain that increase the risk of health problems and stress-related illnesses. Toxic stress can occur when someone lives in persistent poverty or in a high poverty neighborhood with limited resources and increased exposure to negative environmental conditions and can be the mediator between poverty and negative health outcomes in adults.
5. Research with adults in the U.S. found that all five forms of material hardship, including lack of medical care, free food, phone turned off, unstable housing, and problems paying bills, were linked with an increased risk of depression. In children in Turkey, particular indicators of poverty were identified as significant risk factors for high depression rates. Those indicators included living in a single-parent family, rating health as poor, having any disease, deprivation according to the UNICEF Child Deprivation Index (CDI), going to bed or school hungry occasionally, and working to earn money.
6. Everyday discrimination and depression are significantly correlated; increased discrimination is associated with increased depression.
7. There is increasing awareness of the longstanding link between racism–a form of prejudice based on race–and depression. Experiencing racism is associated with poorer mental health, including increased rates of depression, anxiety, and psychological stress.
8. The way individuals are treated in society based on their gender identity and sexual orientation also has implications for their mental health. Research found that women who experienced gender discrimination were significantly more likely to develop depression. LGBTQ individuals experience distress based on their minority status that is different from general psychological stress.
9. Intersectionality examines the impact of multiple underrepresented social identities present in one individual on outcomes of interest, such as depression. For example, LGBTQ individuals who are also part of an underrepresented racial group (e.g., Black individuals) experience more discrimination, with its mental health implications, than their White counterparts.

Now that you've finished reviewing these key takeaways, you can go back and look over the material from this module or move on to take the Module Quiz. Remember to use the information you just learned when you study for the quiz. Good luck!

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